INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Bevacizumab + capecitabine

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00623
Version 3a

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Treatment stages

01 **21-day tablet and infusion cycle**
Tablets + targeted infusion

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	10
04	Sources and further information	16

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Bevacizumab + capecitabine · National Cancer Control Programme (NCCP) 00623

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises. Severe abdominal pain, significant bleeding, sudden weakness or a severe unusual headache also need urgent assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

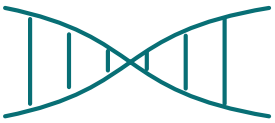
How the treatment works

This combination treats metastatic or unresectable colorectal cancer when an irinotecan- or oxaliplatin-based combination is unsuitable. Its aims may include cancer control, symptom relief and longer survival.

ORAL CHEMOTHERAPY

Capecitabine

Xeloda



Becomes fluorouracil in the body

After you swallow capecitabine, your body changes it into fluorouracil. This interferes with the deoxyribonucleic acid (the cell's genetic material) (DNA) and RNA processes cells use to grow. Normal cells can also be affected, which helps explain mouth soreness, bowel symptoms and changes in blood counts.

Swallowed tablet → Converted to fluorouracil → Cell growth disrupted

What to watch for

Stop tablets and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activities, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed.

ANTI-VEGF TARGETED ANTIBODY

Bevacizumab

Avastin or a prescribed biosimilar



Blocks a blood-vessel growth signal

Bevacizumab binds VEGF, a signal that helps blood vessels grow. Blocking it can restrict the blood supply supporting a tumour. It also affects normal blood vessels and healing, which is why blood pressure, urine and planned procedures matter.

VEGF signal → Bevacizumab binds VEGF → New-vessel signalling is reduced

What to watch for

Report bleeding, a wound that is not healing, severe abdominal pain, new swelling, chest symptoms or a severe headache. Tell the team before surgery, dental procedures or port insertion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and duration

The same medicine can be used for different purposes.

How long?

For advanced disease, this combination continues while benefit and tolerability justify it, with planned reviews.

Alternatives and decisions

Discuss the expected benefit, burdens, reasonable alternatives and the option of supportive care. The best choice depends on your cancer, fitness and preferences.

Understanding the evidence

Ask which evidence matches your cancer, treatment aim and medicines.

In an older population unsuitable for oxaliplatin or irinotecan combinations, adding bevacizumab delayed cancer progression but caused more severe side effects. The capecitabine starting dose in that trial differed from the standard dose in this NCCP regimen. It is not a prediction of your extra lifespan.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Bevacizumab + capecitabine · NCCP 00623

How the treatment fits together

Bevacizumab + capecitabine

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TABLETS + TARGETED INFUSION

21-day tablet and infusion cycle

D1 D14 D15 D21

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

The treating team confirms the aim, duration and next steps. The visual pattern does not replace your prescription.

BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00623 v3a · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Bevacizumab + capecitabine: treatment and checks

Advanced colorectal cancer

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00623	Baseline bloods; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00623	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Blood pressure / urine / wounds NCCP 00623	blood pressure (BP), urine protein and wound/ procedure review	BP before and after treatment; urine protein	Report bleeding, severe headache or abdominal pain urgently	Additional urine tests and specialist review if abnormal	Confirm wound-healing and procedure plan
Tablets / hands and feet NCCP 00623	Confirm written dose, tablet supply and hold instructions	Review adherence, mouth, bowel and skin symptoms	Days 1–14 tablets; Days 15–21 planned break unless team changes it	Do not restart held tablets or replace missed doses without advice	Return unused tablets as instructed; follow persistent symptoms

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Heart checks NCCP 00623	Cardiac history and examination; consider an echocardiogram after chest-wall radiotherapy or prior anthracycline. Further tests depend on symptoms and the clinical plan.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans NCCP 00623	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment
international normalised ratio (blood-clotting test) (INR) if taking warfarin NCCP 00623	Check anticoagulant and baseline INR	If taking warfarin: INR weekly until the warfarin dose is stable, then before each cycle.	Report bleeding or bruising	Adjust warfarin only as prescribed	Continue the agreed anticoagulation plan

What each check is for

1 Confirm your personal plan

Before starting

This combination treats metastatic or unresectable colorectal cancer when an irinotecan- or oxaliplatin-based combination is unsuitable. Its aims may include cancer control, symptom relief and longer survival.

Ask / act: Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.



STEP 2

Taking the tablets

Use the prescribed morning and evening dose within 30 minutes after food on the agreed treatment days. Do not make up a missed or vomited dose. Follow the team's hold instructions for significant diarrhoea, mouth soreness or painful hands and feet, and contact them promptly.

STEP 3

If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.

STEP 4

Getting help between visits

Follow your oncology unit's urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

STEP 5

Operations and dental procedures

Tell both the oncology team and the procedural team that you receive bevacizumab. Holds and restart need an agreed wound-healing plan; do not stop or restart it yourself.

Tests and checks

Monitoring helps the team decide whether treatment can continue.

DPD testing

DPD helps break down fluoropyrimidine medicines. Low activity can cause severe toxicity; testing before treatment helps guide the plan but cannot prevent every serious reaction.

Blood, kidney and liver results

Counts, kidney and liver function are checked before each cycle. Diarrhoea and poor intake can worsen kidney function and make tablets harder to tolerate.

Your symptoms matter

Report bowel frequency, stoma output, mouth soreness, hand-foot pain, chest symptoms and what daily activities you can no longer manage.

Additional checks

Blood pressure, urine protein, bleeding/clot symptoms and healing of wounds are checked with bevacizumab.

Review, recovery and follow-up

Your plan can change as treatment progresses.

Dose changes are individual

A pause or reduction may be needed because of symptoms, blood results or kidney function. The team considers the treatment aim and other options.

After the last dose

Keep reporting new or persistent symptoms after treatment ends. Your team confirms follow-up, scans and recovery support; the timeline is not a forecast.

Prepare for review

Bring questions, symptoms and missed-dose records. Tell the team what matters most to you and ask them to explain the next decision.

Symptoms and supportive care

Bevacizumab + capecitabine · NCCP 00623

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Bevacizumab infusion	 **Day 1** intravenous (IV)
Capecitabine tablets	 **Days 1–14** Twice daily as prescribed
Tablet-free days	 **Days 15–21** No planned capecitabine
Bowel / mouth / skin	 **Days 1–21** May develop or accumulate; report symptoms
Infection / feeling unwell	 **Days 1–21** Seek advice on any day
Blood pressure / bleeding	 **Days 1–21** Bevacizumab effects are not tied to infusion day

Your written plan comes first: Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Act when symptoms occur: Do not wait for the next visit or use the chart to decide a symptom is safe.

Symptoms can build: Bowel, mouth and hand-foot symptoms may accumulate and can continue during tablet-free days.

Across the course: The pattern can change between cycles. Dose changes and treatment breaks are made with the team.

Why these bands stay broad: Treatment bands show the NCCP schedule. Symptom bands are broad teaching illustrations: they do not represent measured daily risks or predict when you will feel well. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00623 v3a · official regimen ↗ · eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule) ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1–14: Treatment days

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription. Use the current tablet label. Swallow whole with water with or within 30 minutes of food; do not crush or cut tablets.

Day 15–21: Planned tablet-free days

Do not take leftover tablets to replace doses missed earlier. Side effects may continue during this interval. Keep the oncology contact plan available and confirm the next review before restarting.

Support around the medicines

1 **Nausea prevention and rescue**

The team agrees a rescue medicine if needed; routine daily anti-sickness prevention is not automatically required for this oral low-risk schedule.

2 **Bowel symptoms**

Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.

3 **Counts and recovery**

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

4 **Skin and mouth care**

Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.

5  **Blood pressure and wound care**

Keep the agreed blood-pressure and urine checks. Report bleeding, poorly healing wounds, new swelling or severe headache promptly.

6 Warfarin monitoring

If taking warfarin: INR weekly until the warfarin dose is stable, then before each cycle.

Taking capecitabine safely

Tablets are active chemotherapy, with a specific schedule.

Meals and doses

Use your current prescription. Swallow tablets whole with water with or within 30 minutes after food. Do not cut, chew or crush them; ask pharmacy if swallowing is difficult.

Missed doses or vomiting

Do not replace a missed or vomited dose. Never double the next dose. Contact the team for repeated vomiting or uncertainty about the schedule.

Pause and restart

Stop tablets for significant diarrhoea, painful mouth/hand-foot symptoms affecting activity, fever or dehydration and seek advice. A toxicity-related pause requires instructions before restart.



Different tablet strengths

Different strengths may be combined to make your dose. A reduction changes tablet numbers: use the new label and do not calculate a change from this page.

Recognise symptoms and act

Do not wait for the next appointment if you feel unwell.

Bowel and hydration

Report diarrhoea or increased stoma output early. Inability to drink, dizziness, very little urine, severe pain or blood in stool needs prompt assessment.

Hands, feet and mouth

Pain, cracking, peeling or ulcers that affect normal activities or eating/drinking need advice. Gentle preventive care helps, but does not replace reporting.

Chest symptoms and infection

Stop capecitabine and seek urgent assessment for chest pain. Fever, shaking chills or sudden illness needs immediate oncology contact, even if the temperature is normal.

Treatment-specific effects

Bevacizumab can cause high blood pressure, urine protein, bleeding, clotting and healing problems. Report severe unusual headache, weakness, significant bleeding or severe abdominal pain urgently.

Daily life and other medicines

Make the plan fit your practical needs.

Eating, drinking and activity

Aim for tolerable meals and fluids; contact the team if intake falls. Gentle activity and rest can be adjusted to your energy. Ask for dietetic help if needed.

Medicine and supplement checks

Check new medicines with oncology pharmacy. Warfarin can require closer INR checks; phenytoin levels may need monitoring. Brivudine is an important prohibited interaction.

Family, work and fertility

Ask about infection exposure, work, intimacy, contraception, fertility and any caring responsibilities. Use the medicine-specific duration of precautions given by your team.

Procedures and travel

Tell the team about planned procedures and travel. With bevacizumab, the timing of surgery and wound healing affects treatment scheduling; never arrange a dose pause yourself.

Questions worth asking

What if I forget a dose or vomit?

Do not take an extra dose. Take the next planned dose at its usual time unless treatment has been stopped; report repeated vomiting and check with the team if unsure.

Why do I need blood tests if I take tablets at home?

Blood counts, kidney and liver results, together with symptoms, guide whether to continue and at what dose.

Do I need a port or a pump?

No take-home chemotherapy pump is used. Bevacizumab needs an IV infusion; your team chooses a cannula or suitable existing line.

What happens after treatment?

Ask for your review and follow-up plan. Continue reporting symptoms after the last dose; further therapy depends on the treatment aim and response.

Try explaining the plan back: Which days do you take tablets? What do you do if you miss a dose? Which symptoms mean stop tablets and call? Who do you contact out of hours?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Bevacizumab + capecitabine · active treatment cycle

Active medicines: Capecitabine + Bevacizumab

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Metoclopramide	10 mg PO up to three times daily as required	Only when needed, as prescribed One option for oral minimal-low risk; check renal function, contraindications, interactions and duration. Do not add all alternatives concurrently.

Additional phase-specific support

- The team agrees a rescue medicine if needed; routine daily anti-sickness prevention is not automatically required for this oral low-risk schedule.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.
- Keep the agreed blood-pressure and urine checks. Report bleeding, poorly healing wounds, new swelling or severe headache promptly.
- Where prescribed under the capecitabine source: loperamide 4 mg initially then 2 mg after each loose stool, maximum 16 mg/day; follow the written local escalation plan. Do not copy the irinotecan high-dose schedule.
- No routine bevacizumab hypersensitivity premedication unless previous reaction or a specific local plan. Review blood pressure, proteinuria, bleeding and wound healing separately from nausea prevention.
- If taking warfarin: international normalised ratio (blood-clotting test) (INR) weekly until the warfarin dose is stable, then before each cycle.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth

Capecitabine

Painful ulcers or reduced intake: stop tablets and call.

2 Chest

Capecitabine

New chest pain: stop tablets and seek urgent assessment.

3 Bowel / stoma

Capecitabine

Significant diarrhoea, increased output or dehydration needs prompt advice.

4 Hands / feet

Capecitabine

Pain or peeling affecting daily activity: stop tablets and call.

5 Blood / infection

Capecitabine

Fever, shivers or sudden illness: contact oncology immediately.

6 Kidneys / urine protein

Bevacizumab

Urine protein is checked; swelling or reduced urine needs advice.

7 Head / blood pressure

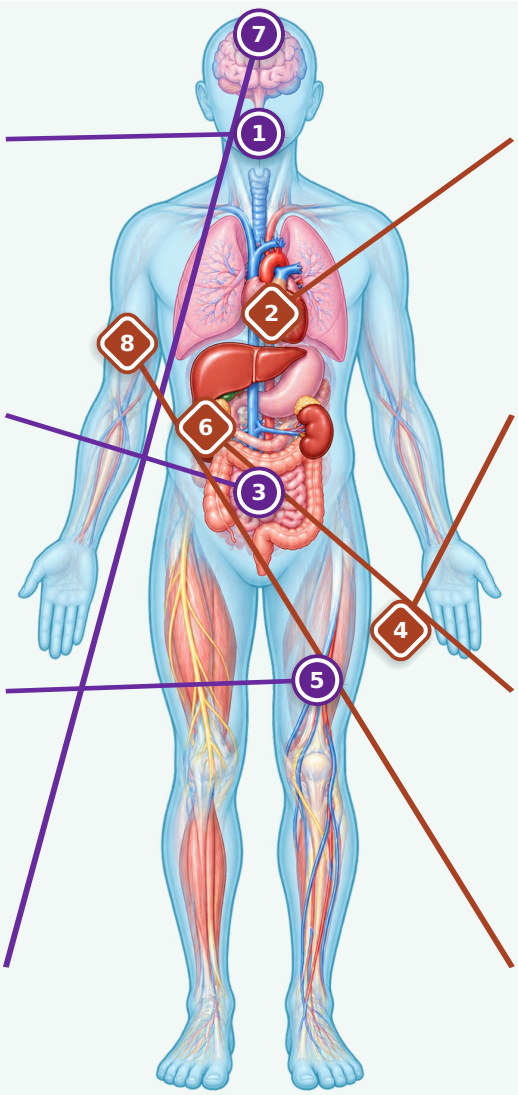
Bevacizumab

Severe headache, visual change or sudden weakness needs urgent help.

8 Bleeding / wounds

Bevacizumab

Report significant bleeding, clots and poor wound healing.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises. Severe abdominal pain, significant bleeding, sudden weakness or a severe unusual headache also need urgent assessment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

Bevacizumab + capecitabine · National Cancer Control Programme (NCCP) 00623

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00623 v3a · official regimen**
   https://healthservice.hse.ie/documents/6686/623_v3a_Bevacizumab_Capecitabine.pdf
2. **eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule)** ↗
   https://www.eviq.org.au/medical-oncology/colorectal/adjuvant-and-neoadjuvant/74-colorectal-adjuvant-capecitabine/patient-information
3. **Capecitabine product information: medicines, interactions and precautions**
   https://www.medicines.org.uk/emc/product/14545/smpc
4. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
5. **AVEX: primary trial report**
   https://pubmed.ncbi.nlm.nih.gov/24028813/
6. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
7. **NCCP Classification V6 (2025), oral minimal-to-low risk Table 5**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf